



Anorectal Disorders

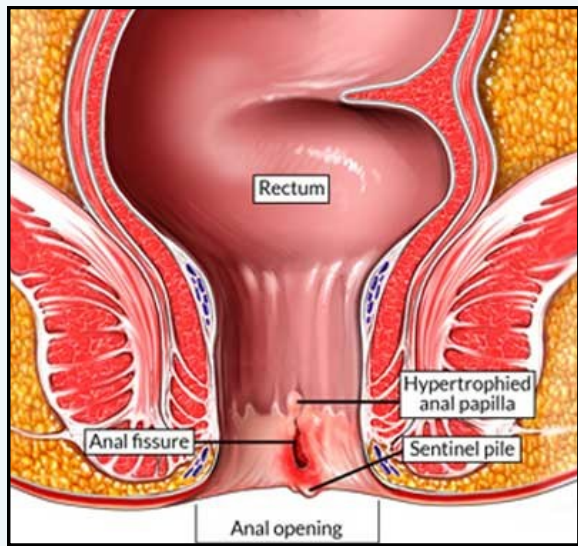
Create a History

When patients complain of rectal pain, bleeding, anal itching, or anal lumps, it's important to start with a detailed history for the patient. Consider the following:

- What's bothering the patient the most (pain, bleeding, itching, hygiene, etc.)?
- Duration
- Exacerbating/alleviating factors
- Bowel history
- Recent life changes (e.g., travel, diet, or medication)

Case 1: Anal Fissure

A 32-year-old male presents with a week of rectal bleeding with bowel movements. He has severe pain and notices bright red blood in the toilet bowl. When he wipes, pain lasts for hours, even after a bowel movement. He has tried over-the-counter hemorrhoid creams, but he feels no improvement of symptoms.



Note. From *Anal Fissure* [Infographic], by Heritage Surgical Group, n.d., Heritage Surgical Group (<https://heritagesurgicalgroup.com/anorectal-surgery/anal-fissure-treatment/>)

It's important to provide expectant management to the patient; fissures may take 2-3 weeks to resolve. If the issue persists, a referral to general surgery is required for examination and consideration of next steps like Botox injection or sphincterotomy.

This is an anal fissure, probably one of the most common things that we see classically. Pain tends to occur after a bowel movement, and the pain may last for hours. Bleeding may also occur.

Patients typically have some sort of inciting event, such as constipation or bouts of diarrhea, which cause trauma.

The first line of treatment is a calcium channel blocker, like nifedipine. The next line of therapy is topical nitroglycerin, which can bring a headache side effect. It's essential to tell patients how to use ointments properly; you do not apply it internally. Typically, you put a bit on the tip of your finger and spread it around the anal verge at least 2-3 times per day. The longer that it's on throughout the day and night, the more it has a chance to work. If the ointment is wiped off, the patient should put more on.

Case 2: Pruritus Ani

A 40-year-old male presents with burning, itching, and irritation after bowel movements. He has tried hemorrhoid creams and wipes without any change in symptoms. He reports having soft stools twice daily.

This is pruritic ani. It's an itchy dermatologic condition usually related to moisture (e.g., microscopic sweat, or stool in the area). Hemorrhoids are less frequently the cause but is a possibility that should be considered if they are prolapsing and producing an irritant mucus.

If the patient does not respond to conservative dermatologic treatments, biopsies may be performed to look for an inflammatory condition.

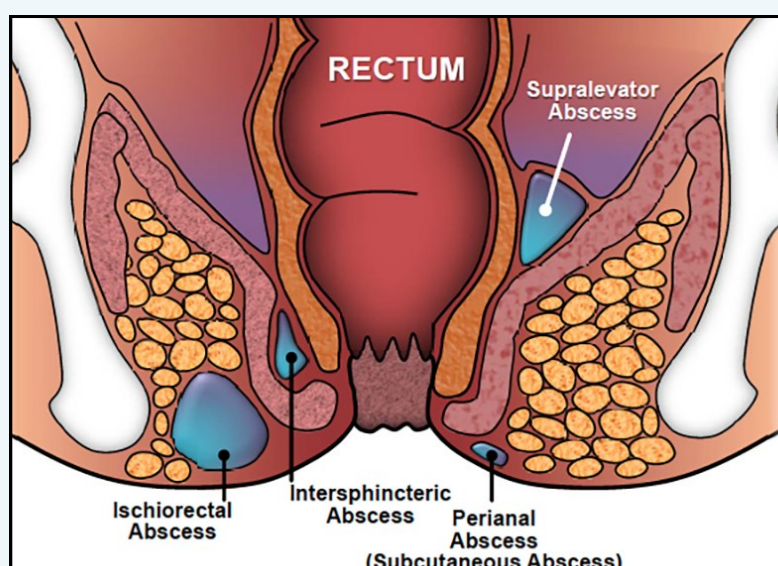
Fiber may help patients to regulate their bowels, but the most important thing is to avoid creams and wipes that may cause an irritant dermatitis. Bidets are recommended to avoid excessive wiping. Barrier ointments like Destin or Calmoseptine may protect the skin.

Case 3: Perianal Abscess

A 24-year-old male presents with a three-day history of worsening anal pain. He went on a long bike ride last week. The patient reports no bleeding, and that he has a bump on his anus that is growing. The area is warm to the touch, red, with some fluctuance. He notes some drops of pus on his underwear and on the toilet paper when he wipes, and he is having some chills.

This is a perianal abscess. Immunosuppression like with HIV or Crohn's is a risk factor. Patients often think they have hemorrhoids, and they may attempt to treat it with hemorrhoid cream without success. Perianal abscesses can be difficult to diagnose virtually, and it is important to consider infection in your differential diagnosis when history suggests an infectious possibility.

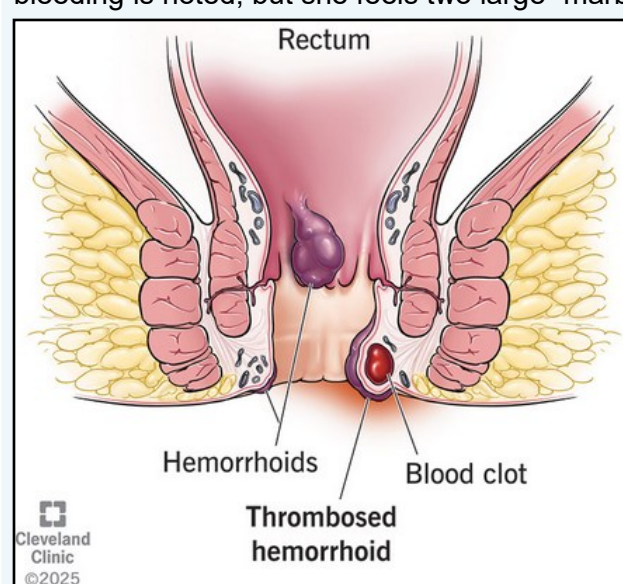
The typical treatment is an incision and drainage at the site of the abscess and this treatment is best administered in the urgent care or Emergency Room (if after hours) setting.



Note. From *Perianal Abscess* [Infographic], by Terence Chua, n.d., Terence Chua (<https://www.drterencechua.com/perianal-abscess-and-fistula>)

Case 4: Thrombosed Hemorrhoids

A 39-year-old female presents with a 2-day history of severe and acute pain in the anal area. No bleeding is noted, but she feels two large "marbles" just outside the anus that are tender when she pushes on them. The pain does not worsen with bowel movements. She has used wipes and creams without significant relief.



Note. From *Thrombosed Hemorrhoids* [Infographic], by Cleveland Clinic, 2025, Cleveland Clinic (<https://my.clevelandclinic.org/health/diseases/thrombosed-hemorrhoids>)

If the patient is in considerable pain and/or has failed conservative management, a trip to the urgent care or emergency room (if after hours) may be in order. In person treatment may include incision and clot evacuation under local anesthesia.

These are thrombosed hemorrhoids, which are very tender. They may be preceded by constipation or diarrhea. It's common that patients diagnosed with this condition have traveled recently, and the condition comes on suddenly.

When there is a tolerable degree of pain with thrombosed hemorrhoids and self-care is elected, they may be treated with sitz baths with Epsom salts to help shrink them. Lidocaine ointment and hydrocortisone ointment topically may also help. Additional treatment options include icing the area, increasing fiber, and utilizing stool softeners and donut pillows. The condition takes a couple of weeks to clear up.

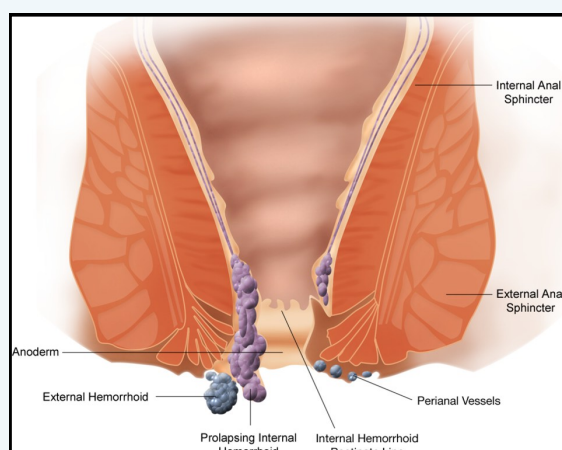
Case 5: Protruding Hemorrhoids

A 53-year-old female presents with a two-year history of bright-red blood with bowel movements. She feels smooth and non-tender "marbles" protruding while wiping and finds it hard to clean them. She notes blood with wiping as well as in the toilet bowl. At age 50, she had a colonoscopy, which demonstrated internal hemorrhoids, but results were otherwise normal.

These are protruding, external, hemorrhoids. Prevention of hemorrhoidal symptoms include adequate fiber supplementation daily. Insoluble fibers, such as psyllium husk (e.g., Metamucil, Konsyl, etc.), work well. Patients may experience bloating with an increased fiber intake, so starting fiber intake slowly is advised. It's also important to have adequate water intake, as fiber doesn't work without water. Advise patients to increase their fiber intake by 5-10 grams per day and increase every few days, titrating to the desired stool consistency and bulk.

Changing the way patients have bowel movements is also advisable. Reducing the time on the toilet and utilizing stools, such as the Squatty Potty, are beneficial.

Don't assume that bleeding is just from hemorrhoids, though. Starting patients with conservative management is advisable. Starting patients on a course of Anusol-HC 1-2 times daily, usually for no more than a week, is the first step in prescription treatment. However, if patients do not respond to conservative treatment, they should be referred to general surgery for a surgical consultation, or to GI for a colonoscopy if they have never had one.



Note. From *Protruding Hemorrhoids* [Infographic], by WikipedianProlific, 2006, Wikimedia Commons (<https://commons.wikimedia.org/wiki/File:Hemorrhoid.png>)

References

- Chua, Terence. (n.d.). *Perianal Abscess* [Infographic]. Dr. Terence Chua. <https://www.drterencechua.com/perianal-abscess-and-fistula>
- Cleveland Clinic. (2025). *Thrombosed Hemorrhoids* [Infographic]. Cleveland Clinic. <https://my.clevelandclinic.org/health/diseases/thrombosed-hemorrhoids>
- Heritage Surgical Group. (n.d.). *Anal Fissure* [Infographic]. Heritage Surgical Group. <https://heritagesurgicalgroup.com/anorectal-surgery/anal-fissure-treatment/>
- WikipedianProlific. (2006) *Protruding Hemorrhoids* [Infographic]. Wikimedia Commons. <https://commons.wikimedia.org/wiki/File:Hemorrhoid.png>